

INDIA' S HEALTHCARE OVERVIEW - CHALLENGES

1. Access and Equity

- Urban-rural divide: Rural areas have shortages of sub-centres, PHCs/CHCs and health staff; urban infrastructure is better but dominated by private high-cost care.
 - Inequitable access across social groups: poorer, marginalised populations face greater barriers to utilisation of services.
 - Out-of-pocket expenditure burden: Many households still pay for outpatient, diagnostics, medicines, and secondary/tertiary care, pushing financial risk.
 - Low public-health spending: India' s government health expenditure as a % of GDP (2.1% in 2021-22) is low relative to many countries.
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2. Health Infrastructure & Workforce

- Weak secondary and tertiary public-sector infrastructure: While primary care built up, district hospitals/CHCs and specialty services are under-resourced.
 - Shortage of health professionals and uneven distribution (especially rural/remote): India is short of doctors, nurses, other cadres; retention in rural areas is poor.
 - Inadequate preventive and primary care focus: Primary health institutions are often oriented more towards curative care; preventive/community health neglected.
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3. Quality, Regulation & Governance

- Fragmented regulatory environment: Standards, accreditation, monitoring vary substantially across regions and public/private sectors.
- Dominance of private sector in tertiary care: Private hospitals often capture publicly-funded insurance payouts, but public hospitals remain underused/under-resourced.
- Focus on hospital-based curative care rather than broader public health/health promotion: Policy tends more reactive than preventive.

4. Financing & Insurance

- Public financing low and health insurance coverage incomplete: Many schemes cover hospitalisation but not outpatient, diagnostics, medicines.
 - Over-reliance on hospitalisation and secondary/tertiary care funding while neglecting outpatient and preventive spending.
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5. Disease Burden and Epidemiological Transition

- Dual burden: Infectious diseases of poverty persist (TB, malaria etc) while non-communicable diseases (NCDs), lifestyle diseases, pollution-related illnesses rise.
 - Need for stronger community health/health-promotion to deal with NCDs and environmental health risks.
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6. Data, Technology & Innovation

- Low investment in medical research & innovation relative to need; limited technology adoption especially in the public sector.
 - Digital health/telemedicine have potential but infrastructural, literacy and regulatory challenges remain.
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7. Implementation & Policy Gaps

- Policy-implementation gap: Many policies exist (e.g., strengthening primary care, insurance coverage) but execution, monitoring and state-level variation weaken outcomes.
- Structural issues: Federal/state coordination, resource mobilisation, human resource allocation, institutional capacity remain weak.

Summary Table (for data-analysis framing)

Challenge Area	Indicative Variables / Data Points
Access & Equity	% rural population with PHC access; doctor/nurse density rural vs urban; % population incurring catastrophic health expenditure
Infrastructure & Workforce	Number of PHCs/CHCs/district hospitals per 100 k; beds per capita; health workforce distribution by geography
Quality & Regulation	% facilities accredited; patient satisfaction/quality indicators; regulatory enforcement rates
Financing & Insurance	Public health spending (% GDP); % population covered by health insurance; share of out-of-pocket health expenditure
Disease Burden	% of burden from communicable vs non-communicable; incidence of TB/malaria vs diabetes/heart disease
Data/Technology/Innovation	% facilities with electronic health records; telemedicine usage; R&D expenditure in health sector
Implementation/Policy Gaps	Time-lag between policy announcement and rollout; % states achieving target metrics; variance across states

Year	Metric (area)	Value
2015	Number of Sub-centres (primary care)	153,655
2015	Number of Primary Health Centres (PHCs)	25,308
2015	Number of Community Health Centres (CHCs)	5,396

Year	Metric (area)	Value
2018 (launch) / 2024 (coverage reported)	Population covered by PMJAY (Ayushman Bharat) — hospitalisation scheme	> 50 crore (≈500 million people)
2021-22	Government/public health expenditure (public spending on health)	2.1% of GDP (2021-22)
2020-21	Government/public health expenditure	1.8% of GDP (2020-21)
2019-20	Government/public health expenditure	1.3% of GDP (2019-20)
2016–2022	Market growth (healthcare sector CAGR)	CAGR 22% (2016 → 2022) — USD 110B → USD 372B (2022)
FY2020-21	Gross direct health insurance premium (market)	Rs. 58,572.46 crore
2020	Medical tourism market value	USD 2.89 billion (2020)
2026 (forecast)	Medical tourism market (forecast)	USD 13.42 billion (by 2026, forecast)
2025 (forecast)	Telemedicine market (forecast)	USD 5.5 billion (by 2025, forecast)
(year reported in Parliament)	Shortage of doctors (national gap reported to Parliament)	≈600,000 doctors short
2021	Employment in healthcare sector (total jobs)	4.7 million people employed (as of 2021)
2017–2022	Job creation in healthcare (increment)	+2.7 million additional jobs between 2017–22

Year	Metric (area)	Value
Policy target (central govt)	Health & Wellness Centres (HWCs) target under Ayushman Bharat	150,000 HWCs planned
Post-COVID (no single year)	Staffing / workforce impact (qualitative numeric trend)	Significant staff shortages; raises in wages/reimbursement pressure on hospitals (no single national aggregate number given)
2016 (review paper)	Quality of care — measurement & variability (qualitative, backed by reviewed studies)	Wide variation: from globally high quality to “unacceptably low” in many facilities; lack of reliable national quality data. (see article for synthesized figures & references)

SOURCES:

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